



Policy

Goals of Patient Care (SCGOPHCG)

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Contents

Scope.....	1
Introduction	1
Risk Statement Patient	2
Definitions	2
Policy Principles.....	2
1. Responsibilities	3
1.1 Medical staff	3
1.2 Members of the Multidisciplinary Team	3
1.3 Patient Support and Clerical Staff	3
2. Clinical Handover	3
3. Procedure.....	4
3.1 Inclusion Criteria	4
3.2 Exclusion Criteria.....	4
3.3 Timeline for GoPC process	4
4. GoPC eForm procedure	4
4.1 GoPC Validity Period	5
4.2 GoPC downtime process	6
4.3 GoPC Amendment Process	6
4.4 Voiding eForms.....	6
4.5 Discharge and Transfer	6
Appendix 1: Filing Goals of Patient Care Form	8
Appendix 2: Supportive and Palliative Care Indicators Tool (SPICT™).....	8

Introduction

This policy provides direction on the implementation of the Goals of Patient Care (GoPC) process. GoPC establishes and documents the most medically appropriate, collaboratively agreed goals of patient care as related to the patient's clinical presentation or in the event of the patient's clinical deterioration.

Risk Statement

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

Definitions

Goals of Patient Care (GoPC) process	GoPC is a process which prompts and facilitates proactive shared decision making between clinicians and patients and / or person responsible / family / carer(s) to ensure treatment provided is aligned to the patient's preferences, needs, values, wishes and cultural safety.
Goals of Patient Care (GoPC) Summary Form	The form provides a summary of the shared discussion to identify, document and communicate the most medically appropriate, agreed goal of patient care that will apply in the event of the patient's clinical deterioration. The agreed goal provides detail of the patient's individual escalation plan including any limitations to treatment for the current admission or during planned extended use (beyond the admission).
Advance Health Directive (AHD)	An instrument recognised under the Guardianship and Administration Act 1990 which records a competent adult's decisions about possible future treatment. Treatment decisions recorded in a valid AHD must be followed in circumstances where the maker of the AHD can no longer make or communicate the decision themselves.
Enduring Power of Guardianship (EPG)	An instrument recognised under the Guardianship and Administration Act 1990 in which a person nominates a competent adult to be an Enduring Guardian.
Advance care planning	The process of planning for future health and personal care whereby the person's values, beliefs and preferences are made known so they can guide decision-making at a time when that person cannot make or communicate his or her decisions.

Policy Principles

Clinicians are encouraged to have open and honest discussions with all patients and their families, so appropriate ceiling of care and end of life wishes can be established.

There is a particular focus on the clinical care of patients with advanced, life limiting conditions.

The GoPC process aims to ensure care is congruent with patient opinion through a common understanding of the goals of care and the individual's escalation plan. It also aims to reduce patient exposure to unwanted / non-beneficial and burdensome treatments.

Management of Advance Health Directives (AHD) is also referred to in this policy due to their synergy. GoPC forms should reflect or take into consideration the patient's AHD or other forms of Advance care planning.

1. Responsibilities

1.1 Medical staff

- The senior medical members of the treating team (registrar level or above) are responsible for completing a timely GoPC discussion for all patients who meet the [inclusion criteria](#).
- They are responsible for encouraging, listening and responding to the patient, family / carer's questions and initiating timely discussions around treatment options, treatment-limiting orders and non-beneficial treatments to enable the patient / the person responsible to be involved in the GoPC decision. The patient's wishes, values and preferences should be accurately reflected in the GoPC process. The treating team should confirm there is not a valid, medically appropriate extended use eform in place at the time of this discussion to ensure this is not superseded inadvertently.
- This documentation may be completed by junior medical officer (JMO) of the treating team (intern or resident) JMO under the guidance and instruction of the registrar or consultant following their GoPC discussion.
- Registrar or consultant details must be captured on the eForm as a record of their involvement in the discussion.
- The published GoPC form must be printed immediately by the completing doctor and placed in the front of the patients work in progress file.

1.2 Members of the Multidisciplinary Team

Nursing, Allied Health, Cultural Safety Officers and other members of the multi-disciplinary team (MDT), although not responsible for completing the GoPC form, are responsible for contributing to the process and providing input into the decisions outlined in the form as necessary.

MDT members should initiate discussion about completing the GoPC process with medical members of the treating team if an applicable patient presents to the ward and GoPC has not been considered.

1.3 Patient Support and Clerical Staff

Clerical staff who manage the input and monitoring of clinical alerts (inpatient and/or outpatient settings) are responsible for the appropriate filing of these documents as per [Appendix 1 flow chart document](#). AHDs must be flagged in WebPAS and filed as a Risk Alert as per HIMS Advance Health Directive (AHD) SOP.

2. Clinical Handover

It is the responsibility of all clinical staff involved in the handover of patient care at any stage of the patient's journey to include information on the details of the GoPC in the handover including to transport staff, staff from other WA Health sites and external private organisations where the handover of clinical care is occurring.

Section 4 must be completed if Goals of Care are to be valid for transport between facilities.

Prior to discharge or transfer to another site, the eForm should be printed and filed in the patients work in progress notes.

SCGOPHCG GoPC forms, with Section 4 completed, are valid for any patients under the medical governance of SCGHOPHCG, while in our care. For eg satellite dialysis units or SCGOPHCG beds at other external sites.

3. Procedure

3.1 Inclusion Criteria

The GoPC process must be implemented as a matter of priority to guide the clinical care of patients with advanced, life limiting conditions. It is at the discretion of the treating clinician to use this form in other groups of patients.

In patients with an AHD, the applicable contents of the AHD should initiate conversations and assist to complete the GoPC form. The AHD should be available for inspection in the medical record. In certain cases, the AHD may no longer represent a medically appropriate management strategy. In these scenarios multi-disciplinary consensus should be sought and a GoPC form should be completed to reflect the most appropriate medical management.

Patients for whom GoPC form is a priority

- Patients unlikely to survive or benefit from a resuscitation attempt
- Patients that meet the Supportive and Palliative Care Indicators Tool (SPICT) criteria. i.e. 2 general and 1 clinical indicator. See [appendix 2](#)
- Any patient who is anticipated to clinically deteriorate
- Patients with advanced, life limiting conditions
- Patients with an Advance Health Directive (AHD)
- Patients who initiate a discussion around ceilings of care

3.2 Exclusion Criteria

Patients for whom GoPC form is not appropriate

- Nil
- For patient's incapable of making an informed decision for GoPC, the discussion may be held with their family/carer/person responsible, as per *Hierarchy of decision-makers for treatment* outlined in the [WA DOH Consent to Treatment Policy](#).

3.3 Timeline for GoPC process

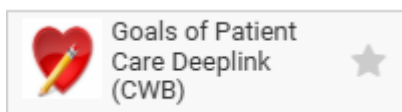
- The GoPC form must be completed as early as possible in the patient journey. This includes, where possible, outpatients or pre-admission clinics.
- For emergency admissions the organisation-wide goal is to complete the GoPC form within 48 hours of admission for relevant patients. However, if clinical deterioration is likely or urgent interventions are planned then the GoPC form should be completed at the earliest opportunity.
- In the absence of a completed GoPC form, the default will, in most cases, be full resuscitation; however, if SMOs present consider cardiopulmonary resuscitation (CPR) to be non-beneficial, then CPR may not be initiated or may be discontinued if already commenced.

4. GoPC eForm procedure

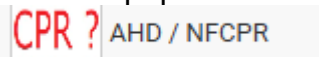
The GoPC eForm is accessible via the App Links tab in iCM and deep link in the digital medical record (DMR). Both platforms take users to Clinical Workbench where the GoPC eForm is completed.

Patient List Results Acknowledge Patient Info Summary Orders Documents Observations Handover eOrder App Links			
Click here to display user guide			
Icon	Abbreviation	Name	Notes
	MHR	My Health Record	
	VNA	VitreView	VNA Viewer
	VNA-Tkn	VitreView - Token Service	VitreView - using Token Services
	INTLCNCT	InteleConnect	InteleConnect Viewer
	MHR settings	My Health Record user settings	
	PACS	PACS Web	PACS Web in context with Patient Launcher
	GoPC	Goals of Patient Care	

eGoPC form via iCM



GoPC deep link in the DMR

- A JMO may complete Section 1 of the GoPC eForm.
- The GoPC discussion with the patient and completion of Section 2, 3 and 4 of the eForm must be conducted by a Registrar or Consultant.
- A JMO may complete all sections of the eForm under the guidance and instruction of the Registrar or Consultant following or during their discussion with the patient/family/carer(s).
- The Registrar or Consultant details must be captured on the eForm as a record of persons involved in the discussion and the completed eForm published.
- The Consultant field is auto filled from WebPAS (patient administration system) and must be checked to ensure it reflects the appropriate Consultants' team undertaking the GoPC discussion. This may be manually overwritten if required.
- Once published, a pdf of the GoPC automatically appears in a pop-up window and can be printed. It will also be accessible via the iCM document list and in the DMR under "Summary" documents. The DMR has a filter which will find GoPC eForms and scanned paper forms
- 
- The published GoPC form must be printed immediately by the completing doctor and placed in the front of the patients work in progress file.
- If completing in an outpatient setting, the patient must be admitted into webPAS first.
- In Section 2, there is provision to upload this document to My Health Record with the patient's full consent.

4.1 GoPC Validity Period

The GoPC is valid for the current admission. A Consultant can endorse the GoPC form for extended use within the same health service beyond the current admission for up to 12 months by completing Section 4. SCGOPHCG GoPC forms, with Section 4 completed, are valid for any patients under the medical governance of SCGHOPHCG, while in our care. For e.g satellite dialysis units or SCGOPHCG beds at other external sites. This is inclusive of hospital home visiting services (HomeLink and HITH).

Section 4 is also used for the purposes of transport between facilities and may be generated in an outpatient setting, planning for future admissions.

4.2 GoPC downtime process

- Should iCM or the GoPC application become unavailable at any time, hard copy GoPC forms will need to be completed.
- A limited number of hard copy GoPC forms are available and should ONLY be used if the eForm is not available via iCM at the time of completion.
- If a hard copy GoPC form is completed due to downtime, this does not necessitate an eForm also being completed.

4.3 GoPC Amendment Process

- If the patient's treatment goals change, a new GoPC form is required to be completed.
- Printed GoPC eForms are NOT to be written on with amended goals.

The revised eForm must be published, printed and filed in the front of the patients work in progress file. All previous versions are visible on iCM.

4.4 Voiding eForms

- In the event an eForm is created in error it can be voided after being published.
- If the GoPC eForm was created incorrectly in the current admission, the medical staff can VOID an eForm.
- If a staff member notices that a GoPC eForm has been incorrectly created (e.g. for wrong patient) during a prior admission/episode of care they should contact the creator of the eForm or the admitting Consultant and confirm the eForm version that needs to be deleted.

Once it is confirmed that a void needs to occur, contact the [SCGOPHCG Systems Support Office](#) requesting a void of the particular GoPC eForm. Email the details of the patient, the specific eForm that needs to be voided, the reason for the void and evidence of confirmation received from the author or admitting Consultant.

4.5 Discharge and Transfer

- Prior to discharge or transfer of the patient to another facility, the printed GoPC form is to be filed in the front of the patient's work in progress file.
- If a patient is admitted/ transferred to SCGOPHCG with a completed and valid GoPC form completed at another site, medical staff are responsible for clarifying the treatment goals with the patient and completing a new GoPC form
- Medical staff are to include GoPC discussions in discharge summaries.
- A completed GoPC form is valid across SCGOPHCG sites.
- If section 4 is endorsed by a Consultant, this is also valid on transfer between sites and readmissions.

Compliance, monitoring and evaluation

Audit reports will be provided to NSQHS Standard 5 and 8 committees.

Monitoring usage via weekly and monthly pre-existing audit tools

Related Documents

[WA DOH Recognising and Responding to Acute Deterioration Policy MP0171](#)

[WA DOH Clinical Handover Policy MP0095](#)

[WA DOH Consent to Treatment Policy MP0175](#)

[SCGOPHCG Recognising and Responding to Acute Deterioration Procedure](#)

[SCGOPHCG HIMS SOP Advance Health Directive](#)

[Filing of GoPC Flow Chart](#)

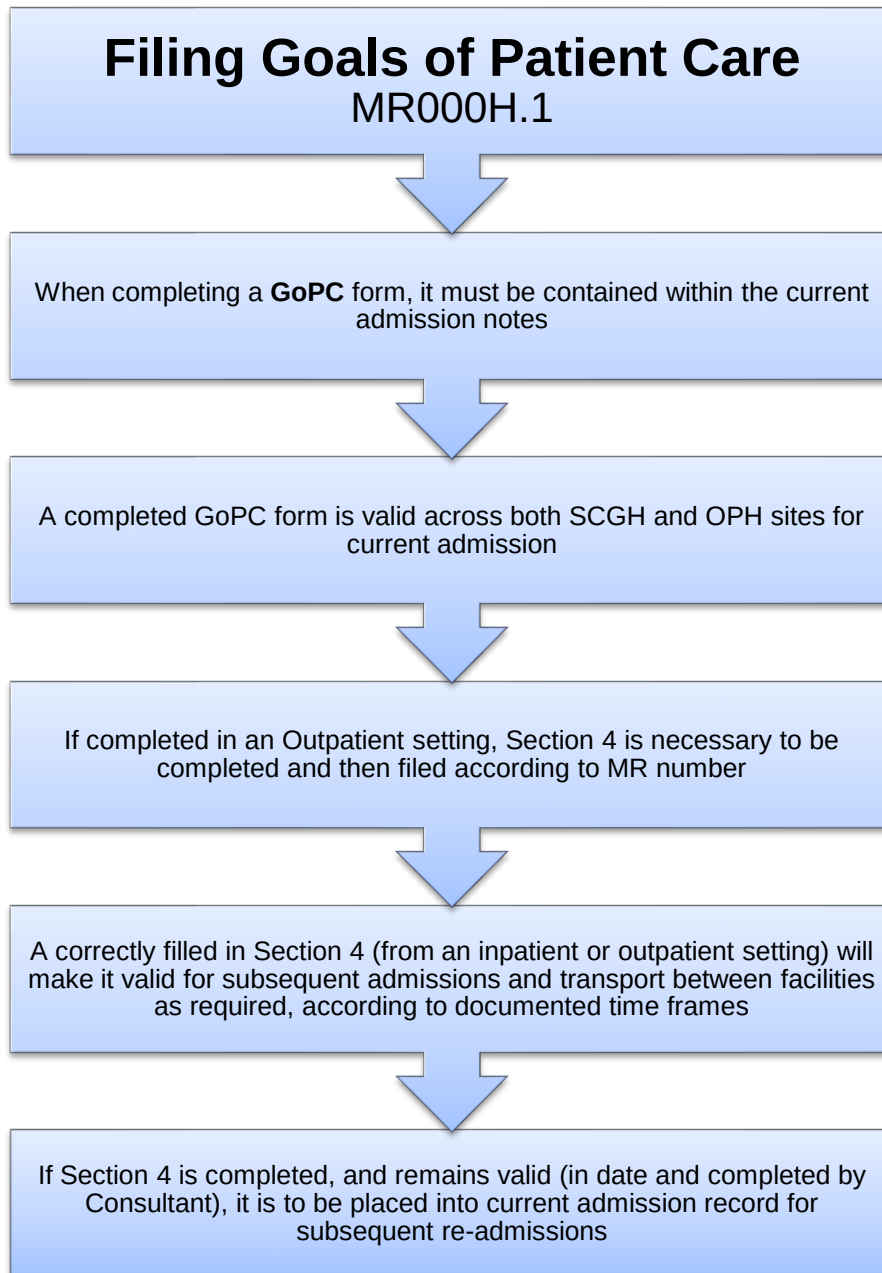
[Goals of Patient Care Summary document \(MR00H.1\)](#)

[SPICT Tool](#)

Websites[Goals of Patient Care NMHS SQP SharePoint page](#)[WA DoH Goals of Patient Care Internet article](#)[Advance Care Planning Australia](#)[NSQHS Standard 6: Communicating for safety](#)**Authorisation & Version Control**

Policy Sponsor:	Area Director Clinical Services				
Policy Contact:	Executive Sponsor Comprehensive Care Committee				
Version #:	1.0	Last Reviewed:	-	Review Due:	24/03/2029
Reference #	HP142	Consumer Engagement:	Yes (Comp Care Committee)	Aboriginal Health ISD #:	5262
Approved by:	Area Director Clinical Services			Date:	06/03/2026
Endorsed by:	SCGOPHCG Policy and Guideline Committee			Date:	24/03/2026
Acknowledgement:	Comprehensive Care Std 5 Committee				
Applicable Standards	NSQHS Standards: Std 1: Clinical Governance  Std 2: Partnering with Consumers  Std 3: Preventing and Controlling Healthcare Associated Infection  Std 5: Comprehensive Care  Std 6: Communicating for Safety 				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Appendix 1: Filing Goals of Patient Care Form



Advance Health Directives (AHD) MR00H

The presence of an AHD document will be entered as a Med Alert into WebPAS and processed according to HIMS SOP Advanced Health Directive (AHD).

An AHD from any source should be reflected in and generate a GOPC Summary.

If an AHD is available on the patients My Health Record, this can be printed and filed in the same manner as the HIMS SOP.



Supportive and Palliative Care Indicators Tool (SPICT)



The SPICT is used to help identify people whose health is deteriorating. Review unmet palliative care needs. Plan current and future care with them.

Look for any general indicators of poor or deteriorating health.

- Urgent or emergency hospital admission(s) or visits.
- Functional ability is poor or deteriorating, with limited reversibility. (eg The person often stays in bed or in a chair for more than half the day.)
- Depends on others more for care due to increasing physical and/or mental health problems. Person's carer needs more help and support.
- Progressive weight loss; remains underweight; low muscle mass.
- Persistent symptoms despite optimal treatment of health condition(s).
- The person (or family) asks for palliative care; chooses to reduce, stop or not have treatment; or wishes to focus on quality of life.

Look for clinical indicators of life shortening conditions.

Cancer

Functional ability deteriorating due to progressive cancer.
Too frail for cancer treatment or treatment is for symptoms.

Dementia or frailty

Unable to dress, walk or eat without help.
Eating and drinking less; difficulty with swallowing.
Urinary and faecal incontinence.
Not able to communicate by speaking; little social interaction.
Frequent falls; fractured femur.
Recurrent febrile illnesses or infections; aspiration pneumonia.

Neurological disease

Progressive deterioration in physical and/or cognitive function despite optimal therapy.
Speech problems with increasing difficulty communicating and/or progressive difficulty with swallowing.
Recurrent aspiration pneumonia; breathless or respiratory failure.
Ongoing disability with worsening physical and/or mental health after a major stroke or multiple strokes

Heart or vascular disease

Heart failure or extensive, untreatable coronary artery disease; breathlessness or chest pain at rest or on minimal effort.
Severe, inoperable peripheral vascular disease.

Respiratory disease

Severe, long term lung disease; breathlessness at rest or on minimal effort between exacerbations.
Persistent hypoxia needing long term oxygen therapy.
Has needed ventilation for respiratory failure or ventilation is contraindicated.

Other conditions

Deteriorating with physical or mental illnesses, multiple conditions and/or complications that are not reversible; best available treatment has poor outcome.

Kidney disease

Stage 4 or 5 chronic kidney disease (eGFR < 30ml/min) with deteriorating health.
Kidney failure complicating other life shortening conditions or treatments.
Stopping or not starting dialysis.

Liver disease

Cirrhosis with one or more complications in the past year:

- diuretic resistant ascites
- hepatic encephalopathy
- hepatorenal syndrome
- bacterial peritonitis
- recurrent variceal bleeds

Liver transplant is not possible.

Review current care and care planning.

- Review current treatments and medication; minimise polypharmacy. Shared decision making about treatment and care.
- Review holistic care – symptoms; emotional, social, financial, spiritual needs. Support families and carers.
- Ask for specialist advice or a review if symptoms or other problems are difficult to manage.
- Agree a current and future care plan with the person/family. Discuss future decision making (e.g. Power of Attorney).
- Record, share, and review care plans.